

Restorative Nursing Handoff Protocol

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Restorative nursing is a nursing-led program that maintains a patient's functional gains after skilled therapy has been discontinued. A strong handoff from therapy to restorative ensures continuity of care, protects the patient from functional decline, and demonstrates interdisciplinary collaboration during survey.

When to Transition to Restorative Nursing

✓ Appropriate to Transition When

- Patient has achieved maximum therapeutic benefit — no further skilled gains expected
- Patient can perform program safely and consistently with nursing-level supervision
- Goals are maintenance-focused and do not require clinical modification
- Patient is medically stable with predictable, low-complexity needs
- Functional level can be maintained without therapist oversight
- Patient and caregivers are trained and understand the program

✗ Not Appropriate to Transition When

- Patient still has active skilled need — therapy should continue or transition to maintenance
- Program requires ongoing clinical modification (Jimmo standard applies)
- Nursing staff have not been trained on the specific program
- Patient is medically unstable or experiencing frequent status changes
- Patient has cognitive deficits that require therapist oversight for safety
- Program is too complex for nursing to safely monitor and implement

Common Restorative Program Types

Range of Motion (ROM)	Passive or active-assistive ROM to maintain joint mobility. Must be performed with proper positioning and documented per session.
Ambulation / Walking	Supervised ambulation at established distance and assist level. Specify device, distance, assist level, and any cues required.
Bed Mobility	Rolling, scooting, sit-to-supine — maintaining independence with technique trained by therapy.
Transfer Training	Maintaining safe transfer technique at established assist level. Specify surface, technique, and equipment required.
Dressing / ADL Program	Maintaining ADL independence or established assisted level. Specify tasks, adaptive equipment, and sequencing cues.
Splint / Orthotic Wearing	Wearing schedule, skin checks, and positioning instructions per therapy recommendation.
Swallowing / Oral Motor	Maintenance swallowing program — diet texture, compensatory techniques, positioning. Must be initiated by SLP.
Cognitive / Reality Orient.	Structured cognitive engagement, orientation cues, memory aids — maintaining established level per ST or OT recommendation.

What the Handoff Must Include — Documentation Requirements

✓ Patient name & diagnosis	Identify the patient and the primary diagnosis driving the restorative need.
✓ Current functional level	Document the patient's functional status at the time of transition — assist level, distance, independence level per program.
✓ Program description	Step-by-step description of each restorative activity — specific enough that any nursing staff can perform it safely.

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✓ Frequency and duration	How often each activity should be performed (e.g., ambulation 2x daily, ROM each shift) and for how long.
✓ Equipment required	Specific devices, adaptive equipment, splints, or positioning supports needed for each activity.
✓ Cues and verbal instructions	Specific cues the patient responds to — what to say, how to grade assistance, what to watch for.
✓ Safety precautions	Weight-bearing status, fall risk level, medical precautions, signs of decline to report.
✓ Decline indicators	Specific changes in status that should trigger a nursing call to therapy for reassessment.
✓ Therapy contact	Which therapist initiated the program and how nursing can reach therapy if questions arise.
✓ Nursing staff trained	Document which nurses or CNAs were trained, what was demonstrated, and their competency.

Roles & Responsibilities

Therapy Responsibilities

- Identify appropriate candidates for restorative before discharge from skilled therapy
- Design the restorative program with specific, achievable activities
- Complete the restorative nursing referral — written, detailed, and in the medical record
- Train restorative nursing staff — demonstrate each activity, observe return demonstration
- Document the handoff clearly — current functional level, program steps, frequency, precautions
- Remain available for nursing questions after transition
- Reassess if nursing reports functional decline or change in status

Nursing Responsibilities

- Obtain physician order for the restorative nursing program
- Enter and maintain the restorative program in the PCC care plan
- Implement the restorative program as written by therapy
- Document each restorative session — activity, duration, patient response
- Monitor for signs of decline identified by therapy
- Contact therapy immediately if patient shows functional decline
- Ensure CNAs performing restorative are trained and competent
- Report any missed sessions and reason in the chart

Monitoring & Reassessment

Therapy reassessment required when

- Patient shows functional decline beyond expected maintenance level
- New medical event (fall, hospitalization, acute illness) affects function
- Nursing reports difficulty implementing the program safely
- Patient refuses restorative consistently — reason must be investigated
- Patient's condition changes significantly — new diagnosis, weight loss, cognitive change

DOR monitoring responsibilities

- Review restorative nursing documentation periodically to ensure program is being implemented
- Confirm restorative patients are not being re-admitted to skilled therapy without clinical reason
- Track restorative program as part of IDT care conference discussions

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- Ensure restorative nursing referrals are in the care plan and current

Monthly Restorative Nursing Meeting — Required

A monthly meeting between therapy and restorative nursing staff is mandatory. This meeting reviews all active restorative patients, updates programs as needed, and ensures nursing staff remain competent. A sign-in sheet must be completed and kept on file as proof of the meeting for State Survey.

Agenda Item	Notes
Review all active restorative patients	
Update or modify any programs per patient status changes	
Address any nursing questions or concerns about implementation	
Identify any patients ready for discharge from restorative	
Identify any patients who may need to return to skilled therapy	

Monthly Meeting Sign-In Sheet

Meeting Date:	_____	Facilitated By:	_____	Building:	_____
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#	Name (Print)	Title / Role	Signature
1			
2			
3			
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#	Name (Print)	Title / Role	Signature
11			
12			

Survey readiness: Surveyors expect to see evidence that therapy and nursing have an interdisciplinary process for transitioning patients from skilled therapy to restorative care. A well-documented restorative handoff with nursing training records is direct evidence of individualized care planning and interdisciplinary collaboration.